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Emotion Work in Ethnography

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Introduction

In this chapter, I explore the impact of emotion work on my research, including how questions of risk, reactivity and privacy were resolved. My thesis concerned the experiences of infertile and sub-fertile couples who were starting fertility treatments. The nature of the study meant that I collected sensitive, highly emotional and intimate data. This presented me with challenges during fieldwork, including how I collected data from participants and how I maintained relationships with participants in the light of the information shared with me. The knowledge that I was likely to stir up unwelcome feelings in both myself and others, to encounter grief and to possibly cause anguish was a form of emotional risk. There was the potential for ‘emotional danger’, that is, threatening and negative feelings experienced by myself and the participants (Lee-Treweek & Linkogle, 2000).

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To investigate the lived experiences of couples, I had designed a longitudinal ethnographic study which used observations in the clinic and repeat interviews. In common with other longitudinal work, the data collection period was long—amounting to eighteen months in the field—with multiple data collection points. Knowledge about the social reality under investigation is formed by the social interactions of the researcher and the researched, which gives rise to field relationships developed over time and shaped by the stance of the researcher (Allen, 2004). Other contributors have written about insider-outsider roles. In this chapter, the focus is on *reactivity* (Hammersley, 2013; Peltó, 2016) by which I mean the effect of the researcher on the field and its participants who may or may not be participants in the research but who inhabit the field. Accordingly, the reflexivity which I employed was the variant Finlay called ‘intersubjective’, that is, concerned with the mutual meanings of the shared field relationship (Finlay, 2002a). When I wrote my research proposal, I had the idea that the ethnographic tension of reactivity might be reduced by greater familiarity between myself and my participants, that repeat interviews might be empowering, for participants and for myself. I also wondered if the research encounters would become easier as the relationship between us developed. However, I found the sustained involvement with the participants in this design brought with it other tensions.

I thought that to research emotion work would be similar to my midwifery experience (Hunter, 2001). It seemed to me that research in highly sensitive areas which necessitated developing ‘empathetic distance’ with participants (Rowling, 1999) is similar to debates about emotion management in midwifery (Hunter, 2001). Would I express empathy and caring differently in a research relationship? When I considered privacy before entering the field, I wondered: how I would access their private accounts of experiences or certain things which could not be said by participants or questions which I could not ask? I did not consider the effect of such private accounts being disclosed through my relationship with participants or the effect on me personally at an emotional level. During interviews, I asked the couples to articulate and reveal their innermost thoughts and feelings about infertility, and I sometimes observed them during the intimacy of the clinic procedures. What follows is a consideration of all these issues in greater detail.

Locating the Self in the Research

As I was engaged in phenomenological research, I understood myself as integral to the social world I studied and located myself accordingly within the research (Finlay, 2002b; Lipson, 1991). Phenomenology takes the subjective perspective as its empirical starting point (Van Manen, 1990). However, gaining an understanding of another's subjective experience is a complex undertaking which is possible only through interpretation. As researchers we exist, 'being-in-the-world' ourselves and are unable to be detached observers; our understanding of phenomena is always based on our background and the historicity of our past experience in the world, whether consciously experienced or not (Koch, 1996). I used participant observation and interviews to collect data and facilitate the emic, insider view but the sense and meanings of participants' experiences were filtered through my understandings (Porter, 2000). Reflexivity is the explicit recognition of this, the process by which I, as researcher, acknowledged my place in the research.

In ethnography, reflexive practice is encouraged by the use of diaries and fieldnotes as they provide a means of critical self-reflection on events and a space for recording emotions, thoughts and accounts of tensions that arise during the research process (Coffey, 1999). Maintaining a balanced insider-outsider relationship has always been a concern of ethnographic method, often referred to as 'keeping the familiar strange and the strange familiar' (Hammersley & Atkinson, 2007, p. 231). Having a reflexive attitude through contemplating their role in a research study is one way the researcher can achieve this. With this, ethnographers have always been committed to seeing the social world from the viewpoint of the subject, (Bryman, 1984) and much ethnographic fieldwork is concerned with obtaining access to the social lives of insiders.¹ Ethnography privileges the position of the researcher through the power they have in representing their participants to the 'outside' (Pillow, 2003). Reflexivity is employed to unmask this: to show that participants are not only telling stories but the stories that the researcher wants to hear.

The literature on reflexivity, even that critical of its practice (Pillow, 2003), suggests that actually 'doing reflexivity' or being a reflexive practitioner is not something which can be easily taught (Finlay, 2008). It is

not simply a case of thinking reflectively about an event or moment, but an active process of being aware of oneself as 'other' and being able to locate oneself in the research. The researcher is the conduit of the research: choosing the questions to ask or avoid and interpreting the data, deciding what is noted and recorded, deciding how questions are framed, what meanings are attached to things that are said or done, and understandings of each and so on. Consequently, I understood that my research would be uniquely mine. My responsibility was not only to give voice to my participants, but also to make my own perspectives, biases and leanings aware to the readers of my outputs. However, as I suggested earlier, this is itself problematic for, as Pillow says, even the notion of 'giving voice' to participants already privileges the superior, knowing voice of the researcher (Pillow, 2003); something which a postmodernist ethnographer attempts to avoid.

The concern of postmodern ethnography is with truth claims and other representations associated with traditional ethnography (Fontana, 2004; Williams, 1990). Today, researchers widen their reflexivity to show, as I have done, their decision-making and how the research was done. In rejecting the idea that a single authoritative voice can provide an explanation for the data, a postmodern ethnographer attempts to disclose the multiple meanings and voices associated with the phenomenon and the representation of the phenomenon (Brewer, 2000). Another goal, again rejecting the idea of 'neutrality' in representation, is for research findings to be understood in terms of the culture being studied and attending to issues of gender relations or power.

Although I identify as a novice researcher with little previous experience of qualitative methodologies I had hoped that being a midwife would be helpful towards my reflexivity, as 'reflective practice' is a common, increasingly mandatory, exercise for appraisal and learning in my profession (Collington & Hunt, 2006). Therefore, some of the ideas of critical analysis, evaluation and self-awareness which this entails (Finlay, 2008) were already familiar to me from processes including annual supervision and practice development. I was less familiar with reflexivity in research practice. Furthermore, the type of reflection dominant in nursing and midwifery practice tends to be a kind of introspective 'thinking about' something, and lacks wider theoretical focus (Finlay, 2008). The

concept of reflexion exists at the other end of a continuum to this simple sort of 'reflection'; reflexion is immediate conscious self-awareness which includes the social contexts of actions (Finlay, 2002b).

A key aspect of being a reflective practitioner or researcher therefore is of 'being in the moment', that is, being able to simultaneously reflect on what is happening (experience) while remaining engaged with whatever the encounter is (Schön, 1983). Whether this is even feasible is debatable as, for example, not all phenomenologists accept that it is possible to split the mind, actions and emotions in this manner because of the embodiment of being-in-the-world. For practical purposes though, it is argued that reflection and action can occur *more or less* simultaneously with practice (Finlay, 2008). In practice, I was uneasily aware of myself in my various roles as both a researcher and a clinic insider (Pellatt, 2003); I was preoccupied with how I positioned myself, what I was wearing, how I should take notes and what other people, both staff colleagues and participants, thought of what I was doing. The participants appeared to be unquestioning of what I was or was not doing in both my observation and participatory roles. Instead it was I who became anxious when research participants confused my research role by asking me questions in consultations or caught my eye (although the doctors did this too during my observations of medical consultations). I was much more focussed and aware of myself being in the clinic than they were. For example, in consultations I consciously kept my head down and moved my chair out of their eye line (in case they tried to include me). And in interviews with participants, I switched the tape recorder off when they wanted advice or wanted me to discuss clinical matters with them. Looking back, I can see that while I was still trying to work out what my research role and stance should be. I was probably overly concerned with establishing distance (Rowling, 1999). One of Rowling's participants admitted they 'hadn't thought about' her role in the research encounter, beyond it being 'professional', and I suspect this would have been similar in my situation. In other words, the boundary between my research and clinical roles was blurred, but seemingly this was of little importance to the participants or other actors in the clinic.

On occasion, participants asked my insider opinion or for my help in sorting out issues they had with the fertility clinic. One example included Cath² who phoned me after her first consultation with a doctor to tell me how unhappy she had been with it. She asked for my advice about what to do. I organised for Cath and her partner Andy to have another appointment, this time with a more senior doctor. Afterwards I was worried that this action would have somehow contaminated my data as it had changed their experience of what the encounter at the clinic had been. With more thought, I reasoned that what I had done was not that unusual as it was merely a normal social interaction, and it is not uncommon in healthcare practice for things to work like this. In the end, I decided that it had given me another perspective on their experience and wrote:

This is a slightly odd situation as I have now become an insider, and of course this might not have been dealt with in the same way if she hadn't known me. Although actually if she had phoned the clinic I think they would have given her a new appointment, maybe not as quickly!

In a later telephone call, Cath and I discussed some of the concerns she had raised with the clinic manager about this first appointment. Together we decided to postpone our research interview until after the rearranged consultation for the reason that 'otherwise she's going to be spending her whole time complaining about the consultation'. In hindsight, this was a missed opportunity as an interview would have given me further insights into what they were feeling. In the end, the second consultation was also enlightening; I felt I'd gained an insider's perspective of the matters which were of most importance to them *and* an awareness of the attitude the clinic staff had towards this couple, as I noted:

Later the administrator starts getting quite irritated with this couple because now they've been asking her more questions! How dare they! It seems they have reached their 'quota' of allocated time and attention and should just be grateful for what they've been given (despite the fact that their consultation wasn't up to par)

As the following examples show there were several other occasions, particularly at the beginning of my fieldwork, where it seems clear that I

was negotiating my identity as researcher at the clinic. Part of this was because some of the nursing staff questioned my new role in the clinic:

One of the nurses saw me later that afternoon and said ‘what are you doing sitting in the waiting room with your pad and clip board today?’ So I told her I was doing a study which involved me observing and interviewing people and she said she hoped ‘I wasn’t observing her doing something wrong’ as she felt “quite nervous”. All this was said in a jokey manner and I don’t think this is a problem as this particular nurse is quite inquisitive and approves of and takes an interest in my studying.

The doctors were curious about what I was doing, but only because they associated observation with audit and assessment. They all questioned me and were concerned with what it meant for them. The most senior consultant asked me ‘did I pass?’ Another made a joke when I first asked him if I could sit in on his consultation:

He said “no!” and then, at my startled reaction, gleefully told me how he likes to give people unexpected answers to questions because [he likes to see] their reactions. Quite odd?’

I suspected this doctor was unsettled by my request, and that his peculiar reply was in some way a projection of his own fear of the unknown. His response was unsettling for me too as it was unexpected, and I found this exchange quite uncomfortable. Later I was able to observe a couple of his consultations, and he always requested ‘feedback’. Another claimed ‘to like it’ (being observed) because of the opportunity it gave for their own learning. The clinicians all equated observation with practice rather than research. Another asked me if I thought he’d been too harsh to the couple because she’d become upset. I told him the purpose of my being there was not to make judgements about him and how he was conducting his consultation, that I was observing the couples. Later, on reflection, I wondered if I had missed a good opportunity to talk to him about this. After all this is ethnography and would have been a good example of a spontaneous corridor conversation and would have told me something about the culture of the clinic. So I suggested that if he’d like to I’d be really interested to hear his thoughts about any other sessions that I

observed with him. He seemed pleased and positive about doing this in the future, especially if they were interesting cases. This remark is quite telling in itself as it demonstrates how routine the consultations are for the clinicians, while my data confirmed that they were extremely important to my participants.

Emotional Risk

Discussing the topic of researcher safety, Lee-Treweek and Linkogle (2000) acknowledge the risks inherent in researching any emotional subject. The threat is from the negative feelings and destabilising state which may be induced from thinking about or hearing accounts that are unsettling and upsetting. The emotional risk inherent in researching sensitive topics is increasingly discussed (Dickson-Swift, James, Kippen, & Liamputtong, 2008), and strategies to minimise such risks, for example, professional supervision and training, have been proposed to counter these. Much of the management of this risk is borne by individuals, who seek an understanding of social realities which have as their defining features emotions and sensitivities. Therefore, this work can be challenging, especially if the topic chimes personally although this was not the case for me. In choosing to research experiences of infertility I was aware that my research participants were often experiencing great hurt and sorrow, akin to grief, and that this would likely arouse uncomfortable feelings for both of us. However, unlike others who have chosen to study this topic (Allan, 1997; Letherby, 2000) I did not identify with my subjects through any immediate personal experience of infertility but I did not view this as a disadvantage.

Letherby argues that she enjoyed greater intimacy and rapport with her respondents as a direct result of her own history (Letherby, 2000). She further suggests that her perceptions and understandings were enhanced because of her commitment and enthusiasm for the topic. Her account illustrates how her own experiences were present in her research, from her choice of topic to interpretation of findings. However, the identity of the researcher as 'other' is complex, and no one identity in the field is privileged. My identity as researcher was multiple; gendered (as a

woman), academic, mother, midwife and fertility researcher all gave me different perspectives and connections with my participants. An instant sympathy with and ability to relate to some participants over others, was likely to be the result of a combination of attitudes, interests and experiences, rather than a single common history. Pillow (2003) critiques strategies of self-reflexivity in which coming to 'know oneself', often through confessional tales or finding experiences in common with the research subjects, become a comfortable act that has the effect of making both researcher and researched more familiar to each other, rather than being used to expose the difficulties and differences inherent in knowing the other (Pillow, 2003). In addition, the introspection and self-awareness necessary for reflexivity to be meaningful should not be allowed to take over the focus of the research; reflexion is not all about the researcher *per se* and, as Finlay (1998) warns, is 'neither an opportunity to wallow in subjectivity nor permission to engage in legitimised emoting' (Finlay, 1998, p. 455). As described earlier, and importantly, reflexion is distinct from reflection because it includes an awareness of 'other', and is not *just* a conscious self-awareness or, in Finlay's words again, 'navel-gazing' (Finlay, 2002a).

The 'swamp' that Finlay describes as the challenge of reflexion is illustrative of its problematic nature (Finlay, 2002a). Phenomenologists are particularly interested in how the 'subject is present in the object' and believe that reflexivity aids in pointing out how differences in our experiences of the same thing are traceable to differences in 'the horizons' through which we experience them (Holloway & Galvin, 2017). My own experiences of reflexivity have been sometimes disconcerting and, as both Finlay (2002a) and Pillow (2003) suggest, 'messy' (Finlay, 2002a; Pillow, 2003). This was because I found it uncomfortable to interrogate my ideas and thoughts; ask questions of myself about why I came to think the way I did or how my understandings of my participants' perspective were realised. It is clear that I used a form of intersubjective reflexivity rather than a broader form of postmodern critique as mentioned earlier (Finlay, 2002a). But as the research developed, my awareness of the broader social critique of care provision for infertile people grew. My main concerns initially were with the relationship between myself and the researched, and in identifying the differences in our situated meanings. I had selected

my topic because of its familiarity and interest to me. Currently my day-to-day working life is based in a fertility clinic; it was while I was new to the clinic that I started thinking about the experiences of the people I observed and met there. I had all sorts of thoughts about the various attitudes, behaviours and actions of both staff and patients, and I had my own ideas about why these might be the way they were. The clinic seemed to be a particular world of which I was not quite a part. As a research nurse, I have a dual role, neither completely 'one of them' [the staff] but also somewhat socialised as a clinic insider. I felt that this was a perfect starting point for ethnographic fieldwork and participant observation much as others have (Arber, 2006).

Accessing the distressing feelings of participants was important as a means of identifying closely with their experiences and providing a genuine 'insider' point of view. I found the emotional distancing inherent in the development of a 'working persona' (Mackintosh, 2007), which I had employed as a practising midwife, helpful in relation to this. The emotional labour that midwives do is often a consequence of their presence in highly emotional, often sad, situations (Hunter & Deery, 2009). Therefore, the experience of being with men and women expressing their feelings was not unfamiliar to me, although that is not to say that I had no personal response to it or had become immune to it. Emotion work is skilled work which has to be learnt despite being undervalued for being perceived as natural to women (Hunter, 2001). Clearly social researchers who are not health service professionals also have interactions with their participants like these and are therefore faced with the similar issues of emotional labour (Dickson-Swift, James, Kippen, & Liamputtong, 2009). The management of emotions is not concerned with preserving researcher detachment or neutrality as in positivist research (Watts, 2007) but to protect the researcher and the researched from negative consequences of emotion work, such as depression and anxiety (Dickson-Swift et al., 2009). Strategies of 'self-care' adopted by social researchers to manage their emotions generally involve methods of distancing, for example reminding themselves that 'the research was not about them'. Nevertheless, it is clear that these sorts of strategies are not always easy to adopt in practice and it is sometimes necessary to utilise more formal support strategies. Supervision, or similar structured debriefing, is useful because the analysis it entails can be transformative; positive feelings are generated through

problem solving, and the disclosure in supervision is another psychological strategy for coping with negative emotions (McLeod, 2009). I found my own emotional coping methods of distraction, exercise and diary-keeping helpful, similar to the distancing strategies and focus on 'own time' used by nurses in emotionally demanding roles (Froggatt, 1995). Other cognitive techniques such as meditation and mindfulness are also said to be useful for some individuals in this respect (McLeod, 2009).

However, ethnographic fieldwork is concerned with emotions and having reactions of anger, sadness, joy, sorrow and various other feelings was to be expected (Coffey, 1999). My experience of fieldwork was no exception. It is worth stating here that I do not believe emotions should be absent from academic work, even though I, like Watts (2007), am unconvinced that it is always possible to integrate them successfully. Like other researchers, I found that I responded emotionally to some of the things I saw or heard, but usually it was after the research episode that I felt able to release my emotions. I do not think that the emotional management that Hochschild (1983) described was at play here as the empathy I offered my participants was genuine—by which I mean authentic—rather than a socially acceptable and expected behaviour (Watts, 2007). The commercialisation of consumer and market research has informed techniques of interviewing which aim to encourage the disclosure of private thoughts, but these methods were not known to me (Duncombe & Jessop, 2002). Instead empathy arose through identification with the emotions of others my participants (Hedican, 2008), either through shared or relational experience (Jones & Ficklin, 2012). As Watts suggests, empathy promotes feelings of trust and rapport in relationships, and includes non-verbal components such as touch (Watts, 2007). Demonstrating oneself as empathetic is difficult because it often consists of an attitude as much as anything else. Here is an example from part of an interview with Sara which I responded to with empathy:

S: I was drained at the end of all that

Gm: Were you?

S: Yes, I was quite—even though I knew what to expect—because my friend had um [pause] oh, who has got pregnant which is really exciting!

Gm: Oh, gosh, lovely

S: Yes from IVF...my friend went through it all with me

Gm: Oh yeah that's right, you'd said hadn't you

S: And so even though I knew what, exactly what, and I'd read the thing, it's still kinda quite ..if someone says it's you it makes it all kind of 'real' and it's a bit, you know. And there's all the permissions and it's all quite 'heavy' stuff isn't it?

Gm: It is heavy stuff [long pause]

Using empathy here was not because I had experience of or memory of a similar situation myself, but perhaps through experience of being in the clinic as a midwife/researcher. I shared an understanding of aspects of Sara's emotions as an understanding of what problems are like for her 'heavy stuff' a weight to be borne (Hedican, 2008).

Similar to other authors (Dickson-Swift et al., 2008), I found transcribing interviews to be emotional despite having conducted the interviews. I had already experienced the dialogue and reacted in the moment to it. I knew before transcribing what I was going to hear, but experiencing the stories again, being focussed and absorbed by the aural account was emotionally powerful. The physical process of transcribing, which involved me listening on headphones, being immersed in the narratives and hearing the same story recounted over and over again (because of my slow typing skills) often made me very sad:

Also it's heart-breaking to listen again to the interview while I am typing it. In between the rational comes little bits and pieces of the emotion that is being suppressed (seems very similar to someone grieving or breaking up with someone). On a personal level it's also moving—I want to be able to help their situation, but of course I can't. The desire to offer advice or encouragement or information is almost unbearable.

And similarly:

This couple are the ones that I find the most moving. Hearing this again makes me remember the feeling I had at the time which was great sadness and an almost overwhelming wish that I could make it alright. I can't.

When I re-read the notes I made after interviewing John and Thea, I can clearly recall the sadness I felt at the time, when I sat in my car after I had interviewed them.

My emotions were sometimes happy as I responded to the changing moods and situations of my participants, for example I was very relieved and happy for Becky and Ali when they had a pregnancy scan that was all fine and struggled to keep my feelings in check:

Waited until I knew pregnancy scan result and it's ok! I am really, really delighted (and relieved). I saw them in corridor to check if I can arrange another interview. Gave Becky a hug and 'congratulations'. (Very unlike me—don't do hugging!) We are both a bit teary.

This was an example where I feel my role as researcher allowed me to cross over the boundaries of my clinical role, as in practice I very rarely hug patients like this.

Reactivity

As I described previously in the first research encounter, couples both told their story and set the scene. I felt that as time went on couples became more guarded about their feelings and I wondered if this was a protective mechanism; that is, to prevent me from continuously picking away at something which was painful, so the research process may have become increasingly difficult for them and for me. The optimism that many expressed in the first interview was often unrewarded, something which may have led them to become less expansive, and more introspective. My data showed many of them tried to stay in the moment and not look too far ahead. As time went on I felt less able to ask probing and searching questions of my participants. For example, after one partner had given me their perspective on an event and the other was quiet I had wanted to draw them out and ask them 'Okay, what did you feel about this (and why is it different?)', but their reluctance to make their feelings known in public was clear. In this instance, I felt further questioning

would be intrusive. Couples were most protective of privacy around their own relationship and as in this case, it was because of the obvious friction between the couple that I had felt unable to probe further. Sometimes individuals told me about their private feelings but less about their differences as couples. It was questions about their relationship which I found most difficult to ask.

Contrary to my hopes that being in the field for a reasonably long period would add to the depth of the data elicited, as our relationships developed into friendships, I was *more* aware of their characters, world-views and sensitivities and was consequently unwilling to disturb the smooth equilibrium of our relationship. Yet the opportunity that repeat interviews gave was the ability for me to revisit subjects with couples, use short-cuts and jokes to signify that I recognised their emotions and, more prosaically, to get clarification about previous statements. I noted for example that Victoria and David were becoming more like friends than research subjects and that we embrace/kiss to greet each other. David became much freer in his conversation; in one of our later interviews for example he was asking about the processes involved in being a sperm donor. When I explained that it took a long time, he quipped was it the 'speed it takes to masturbate or their paperwork? [ha ha!]', something I am sure he would not have said the first times when we knew each other less well. I enjoyed Victoria and David's company but I was not sure how to be a real friend to people with whom I also had a research relationship (Watts, 2007). I was anxious about having to curb our friendship when I left the field and I felt guilty leaving them after, as Maynard (1994) says, 'stirring it all up'. After we had met for the third and final interview I noted that:

I draw a line under this now. I need to complete this interview. They have been lovely and I wish them every happiness. I hope it works out—in between their two personalities they both seem very loving [to each other]—they are still 'trying' so who knows? David walks me back to my car and says 'I hope we see you again' which is really kind but probably unrealistic, and with the best will in the world 'I hope I don't'. Really sad evening.³

As this example demonstrates even those research relationships that include genuine rapport and empathy only really exist for the purpose of

providing data for research. This is uncomfortable to acknowledge, but any research friendship is probably temporary, although this becomes blurred with repeated contact. Duncombe and Jessop (2002) described how their experiences of chance encounters with former interviewees, similar to that I have described above, illuminated the extent to which this occurred. Similar to other researchers I had found it easier to develop rapport with participants who shared demographic characteristics with me, or who had lives closer to my own (Duncombe & Jessop, 2002).

Thea was another woman with whom I had a connection and multiple contacts. Earlier I described that I had found her and John's story very touching, but I found myself withdrawing from engagement with them, particularly Thea, because of a fear that I would be overwhelmed by the sadness that contact with them provoked. This was an example of where my own support mechanisms were important for working through my own feelings of vulnerability that the research relationship with Thea had provoked. The dropping of the clinician boundary put me (the researcher) in a vulnerable position and good support became important here. I started to feel the need for distancing and emotion management. Thea came on her own to meet a consultant after a round of medication had not resulted in pregnancy:

[Thea] is nervous when I get here, 'I don't know why' [she said] and we talked. I said it was 'a bit like a job interview' and that she needs to feel that she'll get something out of it (hence the nerves) which she agrees with. She said she was 'glad to have me there'... I can remember thinking that she seems very vulnerable sitting the other side of the desk to the Consultant today. She knew what he would say and what he was like. I feel a bit let down by John not turning up. I've just come out of the consultation with [Consultant] and Thea and of course she was immediately just crying. Pent up frustration. I have left it up to her again to contact me but she did say it was ok to contact her. She said that in fact it (talking to me) helps and gives her a chance to talk to John. I said I'd give her a call in a month or so. Clearly she was tearful in the consultation. And she's got to fight him as well (I mean John). It was difficult for her that there is nothing left to do, except IVF which I suspect she wants but he doesn't.

[Few weeks later]. I rang Thea as I hadn't heard anything from her since this. No answer. Left an answerphone message. I need to follow up, but clearly I am not looking forward to doing this.

I strongly felt guilty on leaving this research relationship, and I felt as if I had let Thea down. I felt guilt at other times as well:

Had a voicemail from Victoria tonight which I initially felt reluctant to listen to. I had to steel myself to do so; a mixture of worry that she would be pulling out of the research and slight heart-sink that it would be a long and emotional call. Happily however, when I did listen it was just to tell me that she had been offered a cancellation (for surgery) and so was not a problem call. I feel a bit guilty about this, but mainly relieved. However, it's becoming a little tiring having to worry about these calls.

These sorts of feelings are however only present because of the genuine concern and empathy present in our research relationships, and I hope that in small ways the reliable presence of an interested researcher was some consolation to my participants.

Privacy

Infertility is a stigmatised condition and researching it involves asking participants to discuss areas of their lives that are normally kept private. Medical treatments are of a highly personal nature, but often take place in clinics where procedures are technical and have become routinised. I was surprised at how willing couples were to become my research participants, to discuss these familiar aspects of their lives and to have me witness some of these episodes. Sara and Dev were a couple who I had a lot of contact with. The first time that I met them was when they had a consultation with a doctor while I observed. The doctor asked if a medical student could also be present and they said afterwards:

I did have this kind of moment where I was thinking 'oh my goodness how many people can we have in the room, and we are talking about our sex life and it's just, but it was very... it was surreal'!

At a subsequent appointment, I observed Sara having a HyCoSy scan, an intimate clinical procedure involving an internal (vaginal) scan,

carried out by a nurse. This proved to be a painful and distressing experience for Sara:

Today Sara is having HyCoSy scan. The waiting room is empty. She is nervous (clearly), laughing nervously. On her own. So I go in (I wasn't planning on doing this, but seems to be the best thing). AB is doing procedure, assisted by a sonographer. I know them both and they both include me in the procedure. Sara tells them that humour is her coping strategy. She takes shoes off and gets ready. The procedure is in an embryo transfer room, essentially a clinic room. Very sterile, bed with stirrups, no soft furnishings etc. I sit next to the bed and try to be inconspicuous (keep out of their way). AB checks Sara has taken pain killers, explains what she is doing etc, Dev is picking Sara up after the procedure. It's her birthday tomorrow, lots of joking about this. Generally, the procedure is clearly very uncomfortable despite Sara's laughter. She says that 'the thought of it is making her tense up'. She talks a lot through it, AB is concentrating hard as the scan and procedure appears tricky, Sara asks if she is 'playing ball' and then says 'it's nothing to do with me, obviously!' also that it's 'a bit crampy'. My impression is that it's a nasty procedure. Sara says it's like 'period pains and really horrible'. Sara says 'you never really know if you are being a wuss' (she's said this before). We all (the other three women) are in agreement that it's a horrible procedure. Sara now says 'if I need you to stop can I just say?', although she then says it's 'just uncomfortable' (trying hard to be brave?) The sonographer says 'if Ginny was any good she'd hold your hand' which I found quite irritating as I was trying to be invisible and make notes! (She wasn't doing anything either). AB is concentrating as she can't see any dye leaving the tubes although they (the tubes) have stopped spasms now. They talk about what was going to be the next step.

Sara and I talked about this event time and time again in subsequent interviews, which emphasised how important this procedure had been for her. I believe I might have missed its significance if I had not observed it or if it had not been for her repeat comments about what she had felt and experienced at the time. Her main focus was on how painful the procedure had been despite the fact that she had convinced herself that the anticipation would have been worse than the event. Being present while someone is in pain is also a part of the emotional labour that

midwives do, but I found being an observer troubling. It was clear how the routine of clinic procedures, such as this scan, reinforced the detachment shown by clinical staff to the patient's experience. Sara's eloquence about her experience, and my personal investment in her story, gave me a perspective of this event which I would not otherwise have had.

The reflexion on this event is an example of where the differences in our 'horizons of experience' were brought into relief. Sara's thoughts about and anticipation of the HyCoSy scan were crucial in understanding the meaning the experience had for her. Her statement, such as 'it is the not knowing that is worse', is a standard phrase that, for me, carries little meaning beyond stating you are not looking forward to something. My attitude towards the scan was that it was a routine, predictable event, similar to many other procedures familiar to me in my working life. It was enacted on a body (seemingly unconnected from Sara) and with a known purpose and outcome. For Sara, the experience was fully embodied, painful and frightening, with unknown and unexpected consequences in an environment that was alien to her. Embodiment is the idea that human perceptions and experiences are inseparable from physical and social existence or being-in-the-world (Merleau-Ponty, 1962). The belief that the way the world is experienced by people is through their body (all their bodily senses) and not just through consciousness. Understandings of human experiences therefore must relate to their physical presence in the social and cultural world. Sara's recall of the physicality of the experience reminded me about how casually we health professionals can come to view bodily experience as if, for the other, the mind and body are separate. In the fertility clinic, the (female) body is often investigated through a technological lens which further separates this duality. Frequently, as with Sara's scan, clinical staff in the room are focussed on the screen which shows a picture of her insides, rather than the person on the bed at the centre of the experience. In her description, Sara also verbally separates her cervix from her own self when she asks why the procedure is tricky, and this is not an uncommon experience for infertile women who feel their bodies have let them down (Greil, Slauson-Blevins, & McQuillan, 2010).

The received view of intimacy in relationships and friendship is that individuals are more open with people they are close to and more likely to exchange confidences. The research interview then has an inherent

social tension as it breeches social norms in (the UK) society as participants were willing to offer me, a stranger, access to their thoughts and feelings for data collection and analysis. In my notes after the first interview with Thea and John I recognise this tension and the power of the interview and the field relationship:

This is proper stream of consciousness stuff, and it's heart-breaking, but amazing at the same time.

This comment also reminds me of the importance of participant observation as a source of data in ethnography. Narrative accounts rely on the ability of the participants to articulate their thoughts and experiences (Brocki & Wearden, 2006). I found the first interview I had with all my participants was often the one in which they were most likely to try and express how they felt, and it was then that I was able to ask innocent questions to try and establish their initial thoughts about their experiences. During the first interview with John and Thea, I asked her about her feelings being around other people with babies. I noted that I felt awkward asking this question, but felt I needed to ask it. Although afterwards I was reassured that:

Actually when Thea laughs and answers this question, she sounds 'relieved'—probably that she can talk about this which, clearly as from what comes afterwards, she needed to talk about.

I reflected further when I followed-up with questions about her feelings of loss:

I feel a bit sick seeing that I have asked this. It is *so* personal and probing (and obviously I know what the answer is going to be). This is something which I find quite difficult in the interviewing. Sometimes it seems obvious that the couple are not really telling me what they think, but it's quite difficult to get over (my) barrier of asking them the question I need to. I think it's because it's not counselling (and they never signed up for that). However, nobody so far has seemed affronted. Also this section equates to my own experience of loss and bereavement, which is how I relate to what Thea is expressing here.

This note illustrates how my background, attitude and experiences shaped the research encounter that I had with Thea. Another researcher might not have asked this follow-up question, Thea might have recounted her story differently and my interpretation of her grief might have been ascribed to a different emotion. I asked myself to consider whether another couple, David and Victoria, who I had just interviewed had been somehow managing their emotions during the interview as they had been so articulate:

I am mindful of only being able to ask people to express their emotions how they need to, or can. I am very conscious that my interviewing is flawed, however I am also pleased that, in this case at least, the couple have felt able to tell me quite a lot about their private thoughts. I think I have been very lucky to have had such a thoughtful couple to interview.

Another man, Andy, I had wrongly expected to be unable to express his feelings, and this assumption was based on my stereotyping as I was more comfortable at interviewing people more like me. The following observation is quite self-critical and painful to admit:

Partly my prejudice was that Andy was not all that bright, a gym-fixated Neanderthal type man. Actually, I found myself realising that he's actually quite a thoughtful man and my assumptions about them as a couple (slightly bolshie, ignorant etc) were way off. They both surprised me and I feel quite ashamed of myself.

During the interview, I had with Cath and Andy (following their rearranged consultation) they provoke feelings of sadness in me and also I experience uncomfortable and conflicting emotions as I come to see the insider's perspective:

As I am typing this I am struck again about how stream of consciousness this is. Both of them are wounded, they are really bewildered. I also continue to feel (myself) irritated by what they are saying, but also agreeing with it. I can't quite work out if I feel they don't deserve to be treated better because they don't pay lip service to the doctors or give them respect e.g. calling him 'that guy' or expecting a consultant to have phoned them up (As if! How Dickensian am I?!).

The tension I had here was to retain a researcher attitude rather than taking what the participants say as a personal attack on one of my other roles, as a clinic insider, in which I expected the doctor/patient roles as I knew them to be enacted. Of course, within the social setting of the clinic in the early twenty-first century these roles are not as defined as they once were and these participants' attitudes are completely normal. It was through reflecting on what these participants said that made me question my taken for granted assumptions about the setup and conduct of the medical encounters in the clinic. Andy had suggested that the consultant had been 'hard faced, angry and blunt' and that after:

You know, the couple of years of trying, and all your friends getting pregnant and everybody asking you when are you going to have a baby and so there's a lot of emotion wrapped with it so you take all that emotion with you and you, you kind of want someone to be a little bit sympathetic.

A similar re-evaluation occurred during my interview with Katherine and William, a couple who had been due to start in vitro fertilisation (IVF) but had fallen pregnant naturally. They had chosen to use a private acupuncturist before starting the treatment, and attributed their pregnancy to this. My notes describe the confusion I felt during the interview, and I recognise this as stemming from again the tension between the insider-outsider perspectives, but also started my questioning of the biomedical model as the dominant authority of infertility. I reflected feeling:

Strangely ambivalent about this interview. I am really happy for them but it seems amazingly lucky and I think part of me doesn't quite think they should be! But I am starting to doubt the medical aspect of this. I feel much as I did after seeing [X and Y] during Supervision. Perhaps there is something to this snake oil after all. My hackles rise at the presumption of the acupuncturist making clinical decisions, but there again why not? We are all shooting in the dark to some extent here.

At a later point in my research, during data analysis, these comments became very helpful in my findings around the role of the clinic and the patient in the pursuit of treatments for infertility.

Strategies for Reflexivity in the Field

Now that time has passed since the fieldwork stage of my thesis I have been able to reflect on what actually doing reflexivity entailed. I confess to not taking enough heed of this when I had read other researcher's accounts of their ethnographic and observational research. Descriptions of choosing notebooks, making snatched recordings in the sluice and wearing (or not) of uniforms seemed, before I entered the field myself, to be self-evident and the concerns with researcher attitude and emotions somewhat overstated. Now I consider the experiential learning of my observational fieldwork to have given me a revised perspective and with it strategies which I will utilise in my future research projects.

- Make a self-conscious decision to be a reflexive practitioner and to always use diaries and field notes in this pursuit at every research encounter. I found a recording device the most useful way of making short notes in this way.
- Acknowledge the complexities of being a professional who is also a researcher and the difficulties of managing dual identities.
- Don't underestimate the importance of supervision (or other review occasions) as an opportunity for support and debriefing, as well as appraisal.
- Finally, use emotional self-care strategies to provide some perspective when the 'swamp' threatens to overwhelm. These can be cognitive techniques such as meditation or distraction through physical exercise and focus on own time.

Conclusions

Research in 'human science', by its very nature, involves feelings and emotions, and the researcher cannot help but be caught up by this. Emotion work is closely linked to emotional intelligence and both demand qualities, such as self-awareness, empathy, warmth and sensitivity towards others, which enhance human relationships and therefore research encounters with people. Emotions are part of who we are, and help shape the social

interactions through which understandings are formed and as qualitative research is largely about this knowledge, it cannot be separated from it. Indeed, being detached, and by implication, unemotional, are not attitudes that qualitative researchers should aspire to or cultivate. Without a doubt the emotion work in my study, and its impact on my understanding of my participants' experiences, was important and contributed significantly to the development of the findings in the thesis. Very few text books provide guidance on how to handle the emotions that are engendered when carrying out research or how to manage those emotions that arise when the private and professional selves clash (Cain, 2012).

Notes

1. Although in itself this argument presupposes that cultural groups are themselves open or closed and therefore to be understood differently by insiders and outsiders.
2. All participants have been given pseudonyms.
3. As it happens the following year Victoria, David and I met unexpectedly at a social event through a mutual friend. I think all of us enjoyed meeting up again in a different context, and but it did alter the dynamics of the event for me and I suspect that for them it must have been like being at a party with your accountant or priest. When we were chatting I was careful not to allude to the past (or indicate to anyone else our relationship) but Victoria did take five minutes ('as an aside') to tell me her latest news. They had found a suitable egg donor, which is a different outcome to their story than I had expected.

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